SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

FOLFIRI + bevacizumab

Metastatic colorectal cancer · NCCP 00449 · version 6a · 14-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
FOLFIRI + bevacizumab is used for cancer of the colon or rectum that has spread. Treatment aims to control the cancer; the likely benefit and review plan depend on your individual situation. Surgery may be reconsidered in selected cases after multidisciplinary review.

Schedule
Day 1: bevacizumab, irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump. A cycle lasts 14 days; your team confirms the next treatment.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Bevacizumab
ANTI-VEGF TARGETED ANTIBODY

- ☐ High blood pressure / urine protein
- ☐ Bleeding or blood clots
- ☐ Delayed wound healing
- ☐ Rare bowel perforation / fistula
- ☐ Rare serious neurological effects
- ☐ Infusion reaction

Irinotecan
TOPOISOMERASE I CHEMOTHERAPY

- ☐ Early cramps, sweating and diarrhoea
- ☐ Delayed diarrhoea and dehydration
- ☐ Low blood counts / serious infection
- ☐ Nausea, mouth soreness and fatigue
- ☐ Hair thinning or loss
- ☐ Rare serious lung or allergic reactions

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Pump / central-line complications

Confirm DPD testing, anti-sickness medicines, port/PICC care, the 46-hour pump and disconnection arrangements. Explain early cholinergic symptoms and the separate delayed-diarrhoea plan. Discuss blood pressure, urine protein, bleeding/clotting and procedure timing with bevacizumab.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00449 v6a.

SACTCheck v0.84.21
Developed by Dr Paul O'Brien